Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Parotidectomy

Partial or complete removal of the parotid salivary gland (in front of the ear)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Side: ☐ Right ☐ Left

Planned extent: ☐ Superficial ☐ Partial / extracapsular ☐ Total
☐ To be decided during surgery _____

Facial nerve monitoring: ☐ Will be used ☐ Will not be used

Preliminary diagnosis (fine-needle biopsy / imaging result): _____

Decision agreed in advance if the tumour involves the facial nerve: ☐ A nerve branch may be sacrificed ☐ Preserve the nerve ☐ Other _____

1 Your condition and the aim of the procedure

The parotid gland is the largest salivary gland. There is one in front of and just below each ear. The **facial nerve**, which moves the muscles of the face, runs through the gland and divides into branches inside it. Parotid surgery is therefore delicate surgery in which particular care is taken to protect this nerve.

Parotidectomy is most often performed because of a **lump (tumour)** in the gland. The great majority of these lumps (about 80%) are **benign (not cancer)**; the most common are pleomorphic adenoma and Warthin's tumour. Benign lumps can still grow over time, and some (particularly pleomorphic adenoma) can become malignant over many years. Less often, surgery is needed for recurrent infection, stones or a malignant tumour.

The aim of the operation is to remove the lump or diseased gland tissue completely **while preserving the facial nerve**, and to establish the definitive diagnosis by pathological examination.

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2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre. Before surgery the lump is assessed with ultrasound, MRI and/or fine-needle biopsy; these tests give useful preliminary information, but the definitive diagnosis is made by pathology after the operation.

1. **Incision:** The incision usually starts in the natural crease in front of the ear, curves around under the earlobe and continues into a skin crease in the neck. In suitable cases the incision can be kept shorter or hidden at the hairline.
2. **Finding the facial nerve:** The skin and the tissue over the gland are lifted and the main trunk of the facial nerve is identified. If used, a **nerve monitoring** device helps to track the position and function of the nerve throughout the operation.
3. **Removing the lump/gland:** The nerve and its branches are followed carefully while the lump is removed together with a margin of healthy gland tissue. Depending on the position and size of the lump, only the outer part of the gland (superficial), a portion of it, or the whole gland (total) may be removed.
4. **Pathological examination:** The removed tissue is sent to pathology. The final result is usually available within 1–2 weeks.
5. **Closure:** Bleeding is controlled and a thin **drain** (tube) is usually placed to remove any blood or fluid that collects. The skin is closed with fine stitches and a light pressure dressing may be applied.

If the tumour surrounds or has grown into the facial nerve (this mostly happens with malignant tumours), it may be necessary to sacrifice a branch of the nerve, or the whole nerve, in order to remove the tumour completely. This possibility is **discussed with you separately** before surgery. The nerve will not be cut unexpectedly without this having been discussed with you; what will be done if the tumour cannot be separated from the nerve is agreed with you beforehand.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 2–4 hours	Usually 1–2 nights; discharge once the drain is removed

3 Expected benefits and likelihood of success

- Removal of the lump and a definitive diagnosis
- Preventing a benign lump from growing, pressing on the facial nerve and, in the long term, becoming malignant
- In malignant tumours, the first and most important step of treatment
- Ending pain and swelling caused by recurrent infection or stones

For most benign tumours, surgery provides a permanent cure. For pleomorphic adenomas removed with an appropriate technique, the recurrence rate is **below 1–2 in 100 patients**; however, rupture of the tumour capsule or previous surgery increases this rate. A Warthin's tumour can appear as a new focus in the same or the opposite gland. For malignant tumours, the outcome depends on the type and stage of the tumour and further treatment may be needed.

4 Other treatment options

- **Observation:** For some small lumps thought to be benign (for example Warthin's tumour in older patients or those at high surgical risk), regular follow-up with ultrasound/MRI may be discussed. The lump then stays in place and may grow, and a definitive diagnosis cannot be made.
- **Medical treatment:** For infection or stones — antibiotics, plenty of fluids, massage; in suitable cases removal of stones by salivary duct endoscopy (sialendoscopy). Medical treatment is not an option for tumours.
- **Radiotherapy:** Used for malignant tumours that cannot be operated on, or in addition to surgery; it is not the preferred treatment for benign tumours.

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5 What happens if the procedure is not performed?

Without surgery the lump remains and a definitive diagnosis cannot be made. Benign tumours usually grow slowly, but over time they can enlarge, making surgery more difficult and increasing the risk to the facial nerve; pleomorphic adenomas can become malignant over many years (around 5–10% over the years). A malignant tumour, if untreated, will progress and may spread to surrounding tissues and lymph nodes.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Before surgery, blood tests, imaging (ultrasound/MRI) and, if necessary, a fine-needle biopsy are carried out; you will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (please follow the anaesthetist's instructions).
- For male patients, beard and hair around the ear in the incision area may be trimmed on the day of surgery. Depending on the incision, a little hair behind the ear may be tidied.
- Your facial movements are examined and photographed before surgery to record how they work before the operation.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 24–48 hours	There will be tightness, mild to moderate pain and swelling in the operated area, controlled with painkillers. The drain is usually removed on day 1–2 once the fluid decreases. Sleeping with your head raised is recommended. Your facial movements will be checked.
Week 1	After discharge you can return to daily activities. Stitches are usually removed on **days 5–7**. Numbness of the earlobe and the area in front of the ear is noticeable. The pathology result is usually available during this period or shortly afterwards and will be shared with you.
Weeks 2–6	Most people return to desk work within 1–2 weeks. Strenuous exercise and work involving bending and lifting should be postponed for about **2–3 weeks**. If there is temporary facial weakness, it usually begins to improve within weeks; eye protection and facial exercises are recommended.
Months 3–12	The scar gradually fades. Earlobe numbness partly improves. Most patients with temporary nerve weakness recover fully within 6–12 months. Sweating of the cheek while eating (Frey's syndrome), if it occurs, may appear during this period.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values

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based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Pain, tightness and swelling in the operated area
- Numbness of the earlobe and the skin in front of the ear
- A scar at the incision site (fades over time)
- Mild pain on the operated side when opening the mouth or chewing (first few days)

Common conditions (most are temporary or resolve with treatment)

- **Temporary facial nerve weakness** — In about **10–40 in 100** patients (depending on how much tissue is removed and where the lump is), there is temporary weakness at the corner of the mouth, in closing the eye or in raising the eyebrow. It usually affects only one branch and **improves within weeks to months**. If your eye does not close fully during this time, you will need drops, gel and a night-time eye patch to prevent dryness of the eye.
- **Permanent reduced sensation of the earlobe** — The nerve supplying sensation to the earlobe (great auricular nerve) is affected in most operations. Numbness lessens with time in most patients, but some reduced sensation of the earlobe and surrounding area may be **permanent**. This usually does not affect daily life.
- **Frey's syndrome (sweating while eating)** — Sweating and flushing of the cheek on the operated side while eating. Although detected more often with tests, it is noticeable in about **10–20 in 100** patients and usually starts months later. If bothersome, it can be treated with antiperspirant creams or **botulinum toxin** injections.
- **Hollowing of the cheek area** — Depending on how much tissue is removed, a slight hollow in front of the ear and a difference between the two sides may develop. If noticeable, it can be corrected with fat or filler.

Uncommon conditions

- **Fluid collection (seroma) or saliva collection (sialocele) / salivary fistula** — In about **5–10 in 100** patients, fluid or saliva may collect in the operated area; rarely it leaks out through the skin (fistula). It usually settles within a few weeks with needle drainage, a pressure dressing and, if needed, botulinum toxin.
- **Bleeding and haematoma** — In about **1–3 in 100** patients, blood may collect under the skin. If large, it may need to be drained and the bleeding stopped in the operating theatre.
- **Wound infection** — Occurs in **1–3 in 100** patients; it usually settles with antibiotics, and rarely drainage is needed.
- **First-bite syndrome** — Particularly after deep-lobe surgery, a sharp pain at the angle of the jaw lasting a few seconds with the first bites of a meal. It usually lessens with time.
- **Recurrence of the lump** — Although uncommon for benign tumours, the lump may recur in the same area, or a new focus may develop with Warthin's tumour; this may require another operation. In repeat operations the risk to the facial nerve is higher.
- **Thick scar or keloid** — Depending on individual tendency, the scar may thicken; steroid injections or other scar treatments can be used.

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Rare or very rare, but potentially serious conditions

- **Permanent facial nerve paralysis** — For benign tumours at first operation, about **1–3 in 100** patients may be left with permanent weakness of one branch of the facial nerve or, rarely, the whole nerve. This can result in drooping of the corner of the mouth, incomplete eye closure, an asymmetric smile and dry eye; if the eye is not protected, damage to the clear front surface of the eye (cornea) and loss of vision can occur. The risk is higher with malignant tumours, repeat operations and lumps adherent to the nerve. If permanent paralysis develops, further treatment such as eye-protecting medicines, an eyelid weight, nerve repair or facial reanimation surgery may be needed.
- **Final pathology different from the preliminary diagnosis** — Even if pre-operative tests suggest a benign lump, the final pathology may show a **malignant** tumour. In that case further surgery (removal of the remaining gland or the neck lymph nodes), radiotherapy or other treatments may be needed.
- **Large neck haematoma affecting the airway** — Very rarely, a rapidly enlarging collection of blood can press on the neck and cause difficulty swallowing or breathing. This is an emergency: it must be drained in the operating theatre immediately, and rarely the airway may need to be secured (intubation or tracheostomy).
- **Skin blood-supply problems** — Breakdown of the wound edge, particularly below the earlobe, due to reduced skin blood supply; more common in smokers.
- **Change in earlobe shape or altered sensation of the ear** — Rare; caused by the incision or by effects on the nerve.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

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9 When to contact us immediately

- **Rapidly increasing swelling of the neck or cheek**, especially with difficulty swallowing or breathing (emergency — call 112)
- **Bleeding that does not stop** from the drain or wound
- **Fever above 38.5 °C**, increasing redness, pain, warmth or pus from the wound
- **New or worsening weakness** of facial movement
- If you cannot fully close your eye: **redness, grittiness, pain or blurred vision** in the eye
- **Clear fluid leaking** from the wound, or a soft swelling in front of the ear that keeps growing

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Keep the wound dry for the first 48 hours; after that you may take a short lukewarm shower as instructed.
- In the first few weeks, avoiding very sour foods and foods that strongly stimulate saliva may reduce the risk of fluid collection.
- If you have facial nerve weakness, use artificial tear drops during the day and eye gel and an eye patch at night; protect your eye from wind and dust.
- Protect the scar from the sun for at least 6 months (high-factor sunscreen).
- Make sure to discuss the pathology result and the follow-up plan with your doctor; regular check-ups are recommended even for benign tumours.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

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12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

- ☐ I agree that my images are used **only for medical records and treatment follow-up**.
- ☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).
- ☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.
- ☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that the relationship between the facial nerve and the lump can only be seen with certainty during the operation, that the definitive diagnosis will be made by pathology after surgery, and that further treatment may be needed depending on the result.

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☐ Before surgery, it was discussed with me what will be done about the nerve if the tumour has grown into the facial nerve, and my decision has been recorded in the patient details section.

☐ **I GIVE MY CONSENT to the procedure described above.**

☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.